

STI DATABASE MONITORING REPORT

Strategic Information Component

January – June 2025

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Overview

The STI database captures monthly reports on sexually transmitted infections (STIs) from across Jamaica. The data is submitted by four regional health authorities, each responsible for reporting based on the number of health centers within their jurisdiction that offer STI testing and treatment services. The expected number of reports bi-annually month is as follows: Western Region - 108, North East Region - 78, Southern Region - 162, and South East Region - 180, resulting in a national total of 528 expected reports.

Timely and complete reporting is critical to ensure the accuracy of national surveillance data and to inform evidence-based decision-making and policy development for STI management. The database tracks trends and occurrences of STIs using key indicators such as: STI cases by sex and age group, Genital ulcer disease, Genital discharge syndrome, Syphilis, Epidemiological treatment of partners for chlamydia and gonorrhea, Ophthalmia Neonatorum (in infants under 1 month), Epididymo-orchitis, Pelvic Inflammatory Disease (PID), Genital warts, Pediculosis, Scabies, Other STIs and HIV testing.

These indicators support the monitoring of disease patterns and the effectiveness of prevention and treatment interventions nationwide. The database not only supports national-level monitoring but also enables the identification of high burdened populations, which is

Ongoing efforts to enhance data quality, completeness, and timeliness are critical to ensuring the STI surveillance system remains a robust foundation for Jamaica's national public health response.

Reporting Compliance

1.1 Completeness

The reporting period January to June 2025 reflected strong overall performance in the completeness of monthly reports. The Western Region maintained full compliance throughout, achieving 100% completeness in all six months, demonstrating consistent adherence to reporting obligations. The North East Region also performed well in the first half of the period, reaching 100% in January, February, and April. However, there was a minor drop to 92.3% in March and May, followed by a further decline to 84.6% in June, signaling a downward trend that may warrant follow-up. The South East Region reported stable completeness at 96.7% from January to April, but saw a notable decline in May to 70%, before improving to 93.3% in June. This fluctuation suggests a temporary disruption in reporting systems. Meanwhile, the Southern Region showed steady improvement, beginning at 88.9% in January and reaching 96.3% by June, with consistent reporting of 92.6% from February through May (**Figure 1.1**).

Figure 1.1 Completeness of submitted reports by Region

Figure 1.2 Timeliness of submitted reports by Region

Figure 1.2 Timeliness of submitted reports by Region

1.2 Timeliness

Timeliness varied across regions. The Western Region improved consistently from 77.8% in January to 100% in June, leading all regions. The North East Region fluctuated, dropping to 30.8% in March but recovering to 76.9% in May and 69.2% in June. The Southern Region

2.0 STI Surveillance

2.1 Gender

During the reporting period, a total of 22,380 STI cases were reported nationally, with females continuing to account for the majority at 78% (17,454 cases), and males representing 22% (4,926 cases). Female cases remained consistently high throughout the six-month period, peaking in January (3,149) and June (3,105), with the lowest count observed in April (2,699). Among males, case numbers fluctuated modestly, ranging from a low of 735 in February to a high of 885 in April (**Figure 2.1**). While female case trends indicate a cyclical but overall stable burden, male patterns suggest periodic variations, possibly influenced by changes in healthcare-seeking behavior or access to testing services. Despite some monthly variation, the overall data reflect a sustained and disproportionately higher burden of STIs among females.

Figure 2.1 Monthly Trend of STI Cases by Gender

2.3 Genital Discharge Syndrome **2.0 STI Surveillance**

2.1 Gender

During the reporting period, a total of 22,380 STI cases were reported nationally, with females continuing to

2.2 Age Group Breakdown

The highest burden of STI cases was observed in the 50+ age group, which consistently recorded the largest number of cases each month, ranging from 591 in February to a peak of 676 in June. This age group accounted for the highest number of reported infections, highlighting a significant burden among older adults. The 20-24 age group followed closely, with consistently high numbers across the months peaking at 697 in June, the highest monthly count for any age group within the period. Other age groups with substantial case counts included 25-29 and 30-34 years, both showing steady monthly figures ranging between the mid-400s and low 500s. Adolescents aged 15-19 years also represented a notable portion of cases, with counts between 423 and 525 per month, suggesting ongoing transmission risks among young people. In contrast, children aged 0-14 years accounted for very few cases (generally under 100 monthly). Cases with unknown age were minimal overall, except in April, where a notable spike (93 cases) suggests a temporary lapse in data completeness.

Figure 2.2 STI Cases Age Breakdown

2.3 Genital Discharge Syndrome

During the reporting period, Genital Discharge remained the most frequently reported STI syndrome, accounting for consistently high case counts each month. A total of 17,209 cases were recorded from January to June. The highest number was reported in January (3,119), followed by a gradual decline to April (2,663), then a slight rebound in May (2,864) and June (2,988). This trend suggests a sustained and widespread burden of genital discharge across the half-year period. Cervicitis, while significantly lower in volume, accounted for stable numbers across the months, with totals ranging from 153 to 185. The highest number of cases was reported in February (185), and the lowest in March (153). Overall, 1,045 cervicitis cases were documented for the period. The stability in cervicitis reporting may reflect routine screening patterns in women's health services (**Figure 2.3**).

Figure 2.3 STI Syndrome by Month

2.4 Genital Ulcer Disease Syndrome

During the reporting period, Genital Discharge remained the most frequently reported STI syndrome, accounting for consistently high case counts each month. A total of 17,209 cases were recorded from January to June. The highest

2.4 Genital Ulcer Disease Syndrome

Genital Herpes cases remained relatively stable, with a total of 351 cases over the six-month period. The highest number was seen in January (83), followed by a general decline in subsequent months, fluctuating between 46 and 61 cases monthly. The reporting period saw a total of 74 syphilis cases with the characteristic chancre. The highest number was recorded in June (26), marking a sharp increase compared to previous months, which ranged from 6 to 16 cases. This spike may suggest either improved case finding, a localized outbreak, or delayed reporting. Reports also included other STI conditions with lower rates. "Other" infections totaled 38 cases over the period, with the highest number recorded in January (13) and sporadic cases thereafter (**Figure 2.4**).

Figure 2.4 Genital Ulcer Disease Syndrome

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2.5 Syphilis Cases

During the January to June period, Late Latent Syphilis was the most frequently reported stage, with a total of 367 cases. Monthly counts showed a steady increase from 57 in January to a peak of 76 in May, followed by a slight decline to 63 in June. This pattern reflects delayed diagnosis or prolonged asymptomatic infection, indicating possible missed opportunities for earlier detection and treatment. Early Latent Syphilis followed with 196 cases, showing fluctuating numbers across the months. After a slight dip in March (23 cases), there was a notable increase in April (41) and June (46), suggesting improved case finding or changes in screening coverage during those months. Secondary Syphilis accounted for 79 cases, with numbers peaking in March (18). Primary Syphilis had 42 cases, with lower monthly figures ranging from 3 to 11. The number of Tertiary Syphilis cases remained low with 13 total (**Figure 2.5**).

Figure 2.5 Syphilis Cases

2.6 Epidemiological treatment

Partner treatment for Gonorrhoea and Chlamydia (GC/CT) was consistently the most frequently reported intervention during the period January to June. A total of 528 treatments were administered, with monthly counts ranging from 48 in March to a high of 108 in April. In contrast, Epidemiologic Treatment for Syphilis partners was far less common, with only 44 treatments recorded over the same period. Monthly figures ranged from 3 to 13, with a gradual increase observed between January and May, peaking at 13 in May, followed by a slight decline to 11 in June (**Figure 2.6**). The relatively low number of partner treatments for syphilis compared to GC/CT may reflect the lower number of diagnosed index cases or gaps in contact tracing and partner notification.

Figure 2.6 Epidemiological Treatment

2.7 Other STIs and Non-STI conditions

Pelvic Inflammatory Disease (PID) was the most commonly reported STI-associated condition during the six-month period, with a total of 670 cases. Monthly case counts were highest in January (136) and gradually declined to a low of 82 in April, before stabilizing at just above 110 in May and June. Genital Warts ranked second, with 576 cases recorded. While the condition remained relatively stable across most months, there was a sharp decline in March (18 cases). Numbers rebounded in April and May but dropped again in June (72 cases). All Other STIs (non-specified) accounted for a total of 643 cases, with a steady increase observed over time from 59 in January to a significant peak in June (237 cases). Scabies was reported consistently with 204 cases across the period with monthly figures ranging from 25 to 49. Epididymo-orchitis had 29 reported cases, with the highest counts in March and April (8 each). Ophthalmia Neonatorum, an infection occurring in newborns under 1 month, had a total of 19 cases, with peaks in January (5) and May (**Figure 2.7**).

Figure 2.7 Other STIs and Non-STI conditions

2.8 STI Clinic Testing & Surveillance

During the reporting period, a total of 18,716 individuals attended STI clinics nationally. Of these, 4,277 were males and 14,439 were females. HIV testing was conducted for 9,436 attendees, representing an overall testing coverage of 50%. Females had a higher testing uptake, with 70% (6,585) of attendees tested for HIV, compared to 30% (2,851) of males. The HIV positivity yield was also higher among males at 2.3% (57 cases), compared to 0.5% (57 cases) among females. Overall, the national HIV positivity yield stood at 1.2% (114 positive cases).

For syphilis testing, 8,950 attendees were tested, representing 48% of the total. Similar to HIV testing, females had a higher syphilis testing rate at 70% (6,249) while only 30% (2,701) of males were tested. However, the syphilis positivity yield was significantly higher among males at 3.1% (77 cases) compared to 0.7% (49 cases) in females. This resulted in an overall syphilis positivity rate of 1.4% (126 cases).

Table 1. STI Clinic Testing Summary (January- June 2025)

3.0 Outlook

The STI database report for the period January to June 2025 shows that while reporting completeness remained high nationally (95.7%), timeliness of report submissions was suboptimal, particularly in the South East Region, where on-time reporting consistently fell below 10%.

A total of 22,380 STI cases were recorded, with females accounting for 78% of cases. The highest burden of disease was seen in the 50+ age groups, followed by individuals aged 20-24.

Genital discharge was the most commonly reported syndrome, while genital herpes and syphilis continued to be reported, albeit at lower levels. Epidemiological treatment of partners was low, especially for syphilis. HIV and syphilis testing coverage among female clinic attendees was significantly lower than among males, despite females comprising the majority of attendees.

Recommendations

- Implement automated reminders and follow-ups through DHIS2 or email alerts.
- Strengthen Partner Notification and Treatment.
- Enhance contact tracing systems and stress the importance of consistent documentation of epidemiological treatment.